SANJAY GANDHI POST GRADUATE INSTITUTE OF MEDICAL SCIENCES LUCKNOW-226 014

Case Summary & Follow up

Symptom complex

(L) eye panuveitis & retinal vasculitis

(R) eye macular hole - p/surgical repair.

B/L LL inflammatory edema

Mononeuritis multiplex — Sural N. Bx vasculitic neuropathy.

Blood Group———————Tel. No. 7869002835

DOA———————DOO———————Refd. by Dr.———————

Final Diagnosis——— Undifferentiated vasculitis

- Thalassemia trait

Special Notes

WES: Heterozygous mutation in TNFAIP3 (c.1979G>A) (Exon 8)

4/7/25

To Day care

ELNT CYC 3rd dose

Inj. Granisetron 3mg IV stat

Inj. mesna 200mg IV stat

↓

Inj. cyclophosphamide 500mg in 200ml of NS IV over 1hour

↓

Inj. mesna 200mg IV stat

Inj. Granisetron 1mg IV stat

① Tab prednisolone (7am) 12.5mg OD x 2 weeks

↓

10mg OD – To continue

② Tab. Septran DS 1tab every Alternate day

③ Tab. Shelcal 500mg OD

④ Vitamin D3 60,000 IU once monthly.

Brief Clinical Summary

Complaints

Physical findings

01/08/25
Wt - 60.6 kg
Ht - 158.5 cm
BSA - 1.25
1.83

TO DayCare

ELNI cycle 5th dose

Inj. Granisetron 3mg iv stat

Inj. mesna 200mg iv stat

↓

Inj. cyclophosphamide 500mg in 200ml of NS iv infusion over 1 hr

↓

Inj. Granisetron 3mg iv stat

Inj. mesna 200mg iv stat

Adv

① Tab. prednisolone 7.5mg OD x 2 weeks (7am)

↓

5mg OD — To continue

② Tab. Septran DS 1 tab every Alternate day

③ Tab. shelcal 500mg OD

④ VitB3 60/60 IV once monthly

Hospital Stay	Operations/Procedure performed	Complications	Status on discharges	Final Diagnosis (List problems)	Advise at discharge
Review on 18/08/25					

{ - CBC, ESR
- AST | ALT | creatinine | urine R/E }

Dr Mani

18/8/25

Wt 62.5 kgs Ht-157 cms

BP 117/76

Follow

Date	Weight	File NA
		Currently : No Active new tissue
		Neuropathy - improving.
		Intrinsic muscles of hand weakness - improving by 70-80%
		ELNT c/c 6th dose

Hb 11.60 / 3.77L
RBC 4
N70L22

ESR - 13

AST/ALT - 15/13

Creat - 0.7.

Urine R/E - Protein - Nil
RBC - Nil
WBC 2-3

Adv

To Day Cpe

Inj. mesna 200 mg iv stat
Inj. Granisetron 3mg iv stat

↓

Inj. cyclophosphamide 500mg in 200ml of NS iv infusion over 1hr

↓

Inj. mesna 200mg iv stat
Inj. Granisetron 3mg iv stat

Dr Mani

Follow

x 2 month,

Date	Weight	
		Dr Mani

N/A 2 months : 24/10/25

CBC, ESR
AST | ALT | Creatinine
Urine r/m

22-9-25
HT 157 cms
WT - 63.1 kgs
BP 106/71

No Appt

Issues

① Intermittent fever x 2 days : Tmax : 102°F
Also chills and rigors
Afebrile for last 48 hours

② Swelling over both preauricular region
+ pain ⊕ } x 3 days

No dysuria | Blood in urine
No LOA | LOW

O/E : minimal swelling over ⓛ Preauricular region } Likely Parotid swelling
No tenderness

No other swelling palpable

Pharynx - ⓝ

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Rae Bareli Road, Lucknow– 226 014

Name: Mehak Jahan Ara CR. No. 2025489220

Diagnosis: Undifferentiated vasculitis

Investigations Ordered

- ☐ Heamatology
- ☐ Coagulation
- ☐ Cl. Chemistry
- ☐ Urinalysis & Fluids
- ☐ Cytology
- ☐ Bacteriology
- ☐ Serology
- ☐ Plain X-ray
- ☐ Ultrasound
- ☐ CT
- ☐ MRI
- ☐ Nuclear Medicine
- ☐ Immunology
- ☐ Medical Genetics
- ☐ Endocrinology
- ☐ GI Endoscopy

Others

- ☐ .....
- ☐ .....
- ☐ .....
- ☐ .....

℞ 6/10/25

(1) Tab. MMF 500mg 1—0—1 × 7 days

500mg 2 tab BD 2-0-2 To continue

(1 hr before meals)

(2) Stop Azoran

Continue rest

Dr Mani

UNIT - I

Next Appointment on.....at.....in

Prof. Abie Lawrence
Department of Rheumatology
SGPGIMS

(Signature)